LOG BOOK

MD Child and Adolescent Psychiatry
RESIDENCY PROGRAMME

PHASE A (FOR THE YEAR 1 AND 2)

Department of Psychiatry
Bangabandhu Sheikh Mujib Medical University

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*PBL: Problem Based Learning, ** CBLE: Case Based Learning Exercise *** PAY: Phase A Year

Stamp size photo

Personal details of the resident

Name:	
University Registration No.:	
Date of entry in the programme:	
Session:	Roll No.
Father's name:	
Mother's name:	
Date of birth:	National ID No:
M.B.B.S. (Institution):	Year of passing :
Training:	BMDC Reg No.
Contact address Including telephone, email):	
Passport No. (For foreign student)	

Profile of the teachers/supervisors

Name	Designation	Discipline	Specimen Signature	Specimen Initial

General information

The log book (Daily Training Record) is a day to day record of the clinical and academic works done by the resident
There will be two separate log book for Phase A and Phase B. Phase A log book will include the learning activities done by the resident of first three years of training and the Phase B log book will include the learning activities done on 4th and 5th year of training.
The log book will be a pre-requisite for appearing in the phase A summative examination
This log book has to be maintained by all the residents throughout the period of training
The resident will obtain the log book from the course-coordinator of the parent discipline immediately after joining
The resident will make the required entries in the logbook on the same day of the event and get it signed by the supervisor
It is the responsibility of the resident to keep the logbook safe and secured
Entries in the log book will be block-wise
A patient's care means entry to exit of the patient care system.

How to fill the log book

Guidelines for the trainees:
The logbook is to be maintained by all trainees preparing for the MD (Residency).
The logbook is intended for documenting all activities performed by the trainee during the training program.
Trainees are advised to make the required entries on the day of the event.
Completed and duly certified logbook will form a part of the application for appearing in the MD (Residency) examination.

Guideline for the supervisors:
The logbook is a day-to-day record of the clinical and academic work done by the trainee.
Its purpose is to assess the overall training of the candidate and to determine the deficiencies if any, so that they may be corrected.
The supervisors or any other designated trainer (Professor/Associate Professor/ Assistant Professor) who is capable of testing the competencies of the trainees in the specified area should ascertain that the entries of the logbook are complete in all aspect. He should then certify the accomplishment of desired competencies by signing in the appropriate column soon after the activity is conducted.
The course coordinator shall authenticate the entries by signing the certificates. It is suggested that the course-coordinator check the log book at least once a month, so that they can spot any deficiencies or otherwise in the training.

Objectives of the training programme

At the end of the training for MD (Residency) in Psychiatry a candidate should be able to:

- Assess the patient seeking advice for psychiatric problem by:
  - obtaining pertinent history
  - assessing the mental state
  - performing physical examination correctly
  - formulating a working diagnosis
  - deciding whether the patient requires ambulatory care or hospitalization or referral to other health professionals
- Formulate an aetiological hypothesis that covers the predisposing precipitating and perpetuating causes
- Manage patients using the bio-psycho-social of health care by:
  undertaking appropriate biological, psychological and social investigation and interpret the results.
  utilising the knowledge of socio economic, anthropological and cultural factors in dealing with the patients and the family.
  implementing a treatment plan using a evidence based approach
  independently undertaking non pharmacological intervention such as counseling, behavior therapy, supportive therapy etc.
  maintaining follow up of patient at required interval.
  maintaining records of patients.
  independently managing an indoor and outdoor psychiatric facility in a tertiary care health facility.
  participate in evaluation and management plan in liaison psychiatry settings, reproductive health setting, emergency settings, and geriatric and child and adolescent psychiatry cases.
- Make independent foresensic assessment and right appropriate forensic reports
- Organize, monitor and evaluate community psychiatry services using the principle of public mental health and health economics.
- Organize and run structure training programme for post graduate trainees in psychiatry and clinical psychology, undergraduate medical students, fellow health professionals, paramedics and nursing stuffs.
- Use evidence bases and critically evaluate the evidence in literature in relation to his clinical and academic task and decision making.
- Plan, conduct and supervise research activities.
- Use administrative know-how and managerial skills in the smooth functioning of a psychiatry unit and in organizing academic and clinical activities

The objectives of the training may be achieved through different modes of information transfer and learning exercise, some of which are listed below:

Graded responsibilities in patients care and exposure to:

Ward duties
Emergency duties
OPD duties-
Special clinic duties
Liaison psychiatry disease
reproductive health setting
Undergraduate and post graduate setting

Detail case presentations with formulation and short case presentations in ward rounds, outpatient settings and meeting of the mental health team followed by regular feedback by colleagues, seniors and trainers.
Medical audits
Forensic assessment
Journal club
Small group discussion, workshops, seminars, conference, lectures:
Research projects

Links and liaison with primary and secondary care health facilities for the integration of mental health in general health care
Administrative and managerial task assigned

Key to competency levels is given below:
Observer status-1
Assistant status -2
Perform under supervision-3
Perform independently-4
Able to teach and train others and critically evaluate-5

Following table gives an outline of clinical skills to be acquired by the trainee over a period.

A candidate is expected to acquire the laid down level of competence for the following procedures by the end of the each year as given below:

Competency Chart

I. CLINICAL PROBLEM SOLVING AND MANAGEMENT(In Phase A, Year 1,2)						
Sl.No.		Level of Competence				
		Year 1	Year 2	Year 3	Year 4	Year 5
1	Taking pertinent history and mental status examination	1/2/3	4	4	5	5
2	Performing Physical examination	1/2	3	4	5	5
3	Interpreting results of investigations	1/2	3/4	4	5	5
4	Deciding and implementing treatment	1/2	3/4	4	5	5
5	Maintaining follow up	1/2	3/4	4	5	5
6	Communicating effectively with families, doctors, other disciplines	1/2	3/4	4	5	5

II. CLINICAL PROBLEM SOLVING AND MANAGEMENT (In Phase A, Year 1,2)							
Sl.No.		Level of Competence					Number of case
		Year 1	Year 2	Year 3	Year 4	Year 5	
1	Psychometric Tests Administered And Interpreted	1/2	3	3/4	4/5	5	50
2	Counseling Sessions	1/2	3	3/4	4/5	5	30(Complete sessions)
3	Behavioral Therapy / Relaxation Techniques / Non pharmacological Intervention	1/2	2/3	3/4	4/5	5	50
4	Electroconvulsive Therapy	1	2	3	4/5	5	30
5	Problem Based Learning	1/2	2/3	4	4/5	5	25
6	Case Based Learning Exercise	1/2	2/3	4	4/5	5	50
7	Forensic Assessments And Reports	1	3/4	3/4	4/5	5	20
8	Journal Clubs / Seminars	1/2	3	4	4/5	5	25

Rotations for the Residents in Phase A

Total duration: 24 months

- Phase- A training consists of eight Blocks. Duration of each Block is of three months. The last Block (Block-8) will be allotted for formative assessment.
- The remaining 7 Blocks are as follows:

BLOCK	SPECIALITY	DURATION (months)
1	General Psychiatry *	3
2	General Psychiatry *	3
3	General Basic Psychiatry *	3
4	General Psychiatry	3
5	Liaison Psychiatry (Internal Medicine)	3
6	Liaison Psychiatry (Neurology)	3
7	Liaison Psychiatry (Paediatrics)	3
8	Assessment and Examination	3

* Of the seven blocks placement in psychiatry, four blocks are for training in General Psychiatry. The 5th-7th blocks placements are in Liaison Psychiatry. Among these, Block 5 is for training in Internal Medicine and allied. Block 6 is in Neurology, Block-7 is in Paediatrics,

Section A: OPD consultation*

Block…….

Supervisor…….

List only new cases seen in each OPD**

Sl no.	Date	Name of the patient	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

* include all new cases seen/presented/managed by the trainee in a given OPD day
** List only new cases: do not include cases listed elsewhere in the log book

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section A: OPD consultation*

Block…….

Supervisor…….

List only new cases seen in each OPD**

Sl no.	Date	Name of the patient	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

* include all new cases seen/presented/managed by the trainee in a given OPD day
** List only new cases: do not include cases listed elsewhere in the log book

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section A: OPD consultation*

Block…….

Supervisor…….

List only new cases seen in each OPD**

Sl no.	Date	Name of the patient	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

* include all new cases seen/presented/managed by the trainee in a given OPD day
** List only new cases: do not include cases listed elsewhere in the log book

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section A: OPD consultation*

Block…….

Supervisor…….

List only new cases seen in each OPD**

Sl no.	Date	Name of the patient	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

* include all new cases seen/presented/managed by the trainee in a given OPD day
** List only new cases: do not include cases listed elsewhere in the log book

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section A: OPD consultation*

Block…….

Supervisor…….

List only new cases seen in each OPD**

Sl no.	Date	Name of the patient	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

* include all new cases seen/presented/managed by the trainee in a given OPD day
** List only new cases: do not include cases listed elsewhere in the log book

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section A: OPD consultation*

Block…….

Supervisor…….

List only new cases seen in each OPD**

Sl no.	Date	Name of the patient	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
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10.							
11.							
12.							

* include all new cases seen/presented/managed by the trainee in a given OPD day
** List only new cases: do not include cases listed elsewhere in the log book

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block……..
Supervisor…….

SL no.	Date	Name of the pt	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

SL no.	Date	Name of the pt	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
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11.							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

SL no.	Date	Name of the pt	age	sex	Diagnosis	Remarks	Initial of the supervisor
1.							
2.							
3.							
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12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

SL no.	Date	Name of the pt	age	sex	Diagnosis	Remarks	Initial of the supervisor
1.							
2.							
3.							
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5.							
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12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

LIAISON PSYCHIATRY- INTERNAL MEDICINE

SL no.	Date	Name of the pt	age	sex	Diagnosis	Remarks	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

LIAISON PSYCHIATRY- INTERNAL MEDICINE *

SL no.	Date	Name of the pt	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR) (POMR)

Block…….
Supervisor…….

LIAISON PSYCHIATRY- INTERNAL MEDICINE *

SL no.	Date	Name of the pt	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

LIAISON PSYCHIATRY- INTERNAL MEDICINE *

SL no.	Date	Name of the pt	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
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11.							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

LIAISON PSYCHIATRY- NEUROLOGY

SL no.	Date	Name of the pt	age	sex	Diagnosis	Grading	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

LIAISON PSYCHIATRY- NEUROLOGY

SL no.	Date	Name of the patient	Age	Sex	Diagnosis	Remarks	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

LIASION PSYCHIATRY-NEUROLOGY*

SL no.	Date	Name of the patient	Age	Sex	Diagnosis	Remarks	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
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12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

LIASION PSYCHIATRY-PAEDIATRICS*

SL no.	Date	Name of the patient	Age	Sex	Diagnosis	Remarks	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….
Supervisor…….

LIASION PSYCHIATRY- PAEDIATRICS*

SL no.	Date	Name of the patient	Age	Sex	Diagnosis	Remarks	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section B: Case records of the in-patients managed by the resident (POMR)

Block…….....
Supervisor…….

LIAISON PSYCHIATRY- PAEDIATRICS*

SL no.	Date	Name of the patient	Age	Sex	Diagnosis	Remarks	Initial of the supervisor
1.							
2.							
3.							
4.							
5.							
6.							
7.							
8.							
9.							
10.							
11.							
12.							

*includes admitted cases assigned to the candidate. Admitted cases being managed /included in Logbook of the other trainees in same unit is not to be included

Grading: Excellent: 4, good: 3, satisfactory: 2, unsatisfactory: 1

Section C: Procedures

ELECTRO CONVULSIVE THERAPY

Date	Name of patient with age & sex	Diagnosis	*Competency level	Initial of the supervisor

*Key to competency levels:
1. Observer status
2. Assistant status
3. Performed under supervision
4. Performed independently
5. Able to teach others and critically evaluate

Section C: Procedures

PSYCHOMETRIC TESTS ADMINISTERED AND INTERPRETED

Date	Name of the test	Name and age of client	*Competency level	Findings	Initial of the supervisor

*Key to competency levels:
1. Observer status 2. Assistant status
3. Performed under supervision 4. Performed independently
5. Able to train others critically evaluate, and compile psychometric report.

Section C: Procedures

PSYCHOMETRIC TESTS ADMINISTERED AND INTERPRETED

Date	Name of the test	Name and age of client	*Competency level	Findings	Initial of the supervisor

*Key to competency levels:
1. Observer status 2. Assistant status
3. Performed under supervision 4. Performed independently
5. Able to train others critically evaluate, and compile psychometric report.

Section C: Procedures

PSYCHOMETRIC TESTS ADMINISTERED AND INTERPRETED

Date	Name of the test	Name and age of client	*Competency level	Findings	Initial of the supervisor

*Key to competency levels:
1. Observer status 2. Assistant status
3. Performed under supervision 4. Performed independently
5. Able to train others critically evaluate, and compile psychometric report.

Section C: Procedures (Psychotherapy)

Date	Type of Psychotherapy	Name and age of the client	*Competency level	Findings	Initial of the supervisor

*Key to competency levels:
1. Observer status 2. Assistant status
3. Performed under supervision 4. Performed independently
5. Able to train others critically evaluate, and compile psychometric report.

Section C: Procedures (Psychotherapy)

Date	Type of Psychotherapy	Name and age of client	*Competency level	Findings	Initial of the supervisor

*Key to competency levels:
1. Observer status 2. Assistant status
3. Performed under supervision 4. Performed independently
5. Able to train others critically evaluate, and compile psychometric report.

Section D: Emergency encountered (includes Psychiatric Emergency)

Block……

Supervisor……..

Sl. No.	Date	Name of the patients (age & sex), ward, Bed, PIN	Problem/Diagnosis	Competency level	Signature of the supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

*Key to competency levels:

1. Observer status
2. Assistant status
3. Performed under supervision
4. Performed independently
5. Able to teach others and critically evaluate

Section D: Emergency encountered (includes Psychiatric Emergency)

Block……

Supervisor……..

Sl. No.	Date	Name of the patients (age & sex), ward, Bed, PIN	Problem/Diagnosis	Competency level	Signature of the supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

*Key to competency levels:

1. Observer status
2. Assistant status
3. Performed under supervision
4. Performed independently
5. Able to teach others and critically evaluate

Section E:
JOURNAL CLUB, SEMINERS, PBL & CBLE

Supervisor…….

Sl. No.	Date	Topic/article	Source/ Re-source person	Performance level	Signature of supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

Key for performance of the candidates:
Attended – A Presented himself – PH

Section E:

JOURNAL CLUB, SEMINERS, PBL &CBLE

Block…….

Supervisor…….

Sl. No.	Date	Topic/article	Source/ Re-source person	Performance level	Signature of supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

Key for performance of the candidates: Attended – A Presented himself – PH

Section E:
JOURNAL CLUB, SEMINERS, PBL & CBLE
Block…….

Supervisor…….

Sl. No.	Date	Topic/article	Source/ Re-source person	Performance level	Signature of supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

Key for performance of the candidates:
Attended – A Presented himself – PH

Section E:
JOURNAL CLUB, SEMINERS, PBL & CBLE
Block…….
Supervisor…….

Sl. No.	Date	Topic/article	Source/ Re-source person	Performance level	Signature of supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

Key for performance of the candidates:
Attended – A Presented himself – PH

Section E:
JOURNAL CLUB, SEMINERS, PBL & CBLE
Block…….

Supervisor…….

Sl. No.	Date	Topic/article	Source/ Re-source person	Performance level	Signature of supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

Key for performance of the candidates:
Attended – A Presented himself – PH

Section E:
JOURNAL CLUB, SEMINERS, PBL & CBLE
Block…….

Supervisor…….

Sl. No.	Date	Topic/article	Source/ Re-source person	Performance level	Signature of supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

Key for performance of the candidates:
Attended – A Presented himself – PH

Section F: Case presentation in clinical meeting, grand & ward round

Block…….

Supervisor…….

Sl. No.	Date	Patient's name (age & sex) Ward/bed, PIN	Diagnosis/ Problem	Level of performance	Signature of supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

Key for performance of the candidates:

Attended – A Presented himself – PH

Section F: Case presentation in clinical meeting, grand & ward round

Block…….

Supervisor…….

Sl. No.	Date	Patient's name (age & sex) Ward/bed, PIN	Diagnosis/ Problem	Level of performance	Signature of supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

Key for performance of the candidates:

Attended – A Presented himself – PH

Section F: Case presentation in clinical meeting, grand & ward round

Block…….

Supervisor…….

Sl. No.	Date	Patient's name (age & sex) Ward/bed, PIN	Diagnosis/ Problem	Level of performance	Signature of supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

Key for performance of the candidates:

Attended – A Presented himself – PH

Section G: Presentation/attendance in seminars, symposium/workshops, conferences

Block…….

Supervisor…….

Sl. No	Date	Topic/article	Source/Re-source person	Performance level	Signature of supervisor
1.					
2.					
3.					
4.					
5.					
6.					
7.					
8.					
9.					
10.					

Key for performance of the candidates:

Attended – A Presented himself – PH

Section H. Lectures, Tutorials, Group discussions attended

Block……

Supervisor……

Sl. No.	Date	Topic	Teacher's name	Signature

H Lectures, Tutorials, Group discussions attended

Block……
Supervisor……

Sl. No.	Date	Topic	Teacher's name	Signature

H. Lectures attended, Tutorials, Group discussions attended

Block……
Supervisor……

Sl. No.	Date	Topic	Teacher's name	Signature

H. Lectures, Tutorials, Group discussions attended

Block……
Supervisor……

Sl. No.	Date	Topic	Teacher's name	Signature

H. Lectures, Tutorials, Group discussions attended

Block……....
Supervisor……

Sl. No.	Date	Topic	Teacher's name	Signature

H. Lectures, Tutorials, Group discussions attended

Block……
Supervisor……

Sl. No.	Date	Topic	Teacher's name	Signature

Section I. Interpretations of lab data and investigation reports

Block…….
Supervisor…….

Sl. No.	Items	Abnormality	Interpretation	Signature of supervisor

Section I. Interpretations of lab data and investigation reports

Block……..
Supervisor…….

Sl. No.	Items	Abnormality	Interpretation	Signature of supervisor

Section J. Activities in the Outreached Centres

Block……..
Supervisor…….

Sl. No.	Date	Name of the Institution	A brief account of task/work done	Remarks	Signature of supervisor

Section K. Other Activities

Block……..
Supervisor…….

Sl. No.	Date	Name of Activities	Remarks	Signature of supervisor

Section L. Leave record

Duration	From	To	Reason	Signature of Supervisor

Signature of the Supervisor

Signature of the Chairman

Section M. Summary Records: PAY1

Events	Performed					Signature of the Year-manager1
	Block 1	Block 2	Block 3	Block 4	Total	
A) OPD consultation						
B) Case records of the in-patients						
C) Procedures						
D) Emergency encountered						
E) Journal clubs						
F) Case presentation in clinical meeting, grand & ward round						
G) Presentation/attendance in seminars, symposium/workshop, conferences						
H) Lectures attended						
I) Data Interpretations						
J) Activities in outreached centers						
K) Other Activities						

Section N. Summary Records: PAY2

Events	Performed					Signature of the Year-manager2
	Block 5	Block 6	Block 7	Block 8	Total	
A) OPD consultation						
B) Case records of the in-patients						
C) Procedures						
D) Emergency encountered						
E) Journal clubs						
F) Case presentation in clinical meeting, grand & ward round						
G) Presentation/attendance in seminars, symposium/workshop, conferences						
H) Lectures attended						
I) Data Interpretations						
J) Activities in outreached centres						
K) Other Activities						

Section O. Summary Records: Phase A completion

Events	Performed				Signature of the Course coordinator
	PAY1	PAY2	Total	Comments	
A) OPD consultation					
B) Case records					
C) Procedures of the in-patients					
D) Emergency encountered					
E) Journal clubs					
F) Case presentation in clinical meeting, grand & ward round					
G) Presentation/attendance in seminars, symposium/workshops, conferences					
H) Lectures attended					
I) Data Interpretations					
J) Activities in outreached centres					
K) Other Activities					

Section P. **CERTIFICATE OF ACCURACY OF PHASE-A LOG BOOK**

I certify that the information contained in the Logbook (Daily Training Record) is a true and accurate record of my training experiences.

Trainee's signature: ..... Date: .....

Section Q. CERTIFICATION OF PHASE-A LOG BOOK

I, to the best of my knowledge, certify that

Dr.
……

has satisfactorily completed this logbook as required by the university

…….
Signature of the Course coordinator

Name:

Discipline:
Date:

Signature of the Supervisor